



Guidelines for HIV Pre-Exposure Prophylaxis (PrEP)

Eligibility Criteria for PrEP

- Patients should be adults or adolescents weighing at least 35 kg, who are at risk for sexually-acquired HIV infection.
- Patients should have no laboratory evidence of HIV.
- Patient should understand that PrEP is not a guarantee against contracting HIV.
- Patients should understand the possible risk of toxicity (renal, bone, etc.) and the risk of potential resistance development should they acquire HIV.
- Patients should have creatinine clearance ≥ 60 mL/min.
- Female patients should not be pregnant and are actively employing effective contraceptive methods prior to starting PrEP (unless PrEP is part of a safer conception strategy for serodiscordant couples).



Note. From Pre-Exposure Prophylaxis [Photograph], by NIAID, 2016, National Institute of Allergy and Infectious Diseases. https://commons.wikimedia.org/wiki/File:Pre-Exposure_Prophylaxis_%28PrEP%29_%2832514377531%29.jpg

Exclusion Criteria for PrEP

- Patients who exhibit signs or symptoms of acute HIV.
- Patients who are Hepatitis B Surface Antigen (HBsAg) positive. (While these patients may be considered for PrEP, patients with active chronic Hepatitis B should be considered for treatment under current Hepatitis B guidelines.)
- PrEP should be used with caution in patients with a history of pathologic bone fractures.
- PrEP should be used with caution in patients with significant pre-treatment proteinuria.
- Patients unwilling to adhere to a daily medication regimen.

Beginning PrEP

1. Providers should document that the patient is aware that PrEP is only one part of an ongoing sexual risk reduction program that should include the use of condoms and other effective prevention strategies.
2. Providers should prescribe one (1) tablet of tenofovir-emtricitabine fixed dose combination (trade name Truvada®) daily.
3. Providers should prescribe no more than a 30-day supply for the first 2 months of therapy. After the first two months, the prescription can be increased to a 90-day supply at the discretion of the provider.
4. Patients new to KP who have already been established on PrEP by a non-KP provider may have a 90-day supply dispensed at the discretion of the provider.
5. Each refill should be refilled for only one time, to assure compliance monitoring.
6. Medication compliance will be followed by HIV pharmacist or appropriate personnel designated for this purpose.
7. The use of PrEP should be noted in the KP HealthConnect (KPHC) problem list.
 - This can be accomplished by putting “HIV Pre-Exposure Prophylaxis” in the KPHC problem list (code 555281).
8. Patients should be instructed that PrEP is exclusively for their own use, and it should not be shared with others. Consider monitoring refills for adherence or too-early refills.
9. Risk-reduction counseling and PrEP medication adherence counseling will be provided at the initial visit.
 - Patients with active substance use should be encouraged to seek substance reduction counseling and provided such information.

Centers for Disease Control and Prevention

MMWR | **PrEP: An Essential Tool to End HIV**

Pre-exposure Prophylaxis (PrEP)

- **A powerful way to prevent HIV**



- **Used daily, PrEP dramatically reduces risk of acquiring HIV through sex**

More PrEP Use is Needed



PrEP use up from 6% to 35% among MSM**

PrEP use still too low, especially among Black and Hispanic MSM

Healthcare Providers Can Help End HIV!

- ✓ **Test for HIV**
- ✓ **Assess patient risk**
- ✓ **Prescribe PrEP as needed**
- ✓ **Use CDC resources***



Footnote: Data from CDC's National HIV Behavioral Surveillance (NHBS) (20 cities) as reported in Finlayson et al. MMWR 2019. [bit.ly/CDCVA29](https://www.cdc.gov/hiv/pdf/risk/prep/cdc-hiv-prep-guidelines-2017.pdf)

*PrEP clinical practice guidelines. <https://www.cdc.gov/hiv/pdf/risk/prep/cdc-hiv-prep-guidelines-2017.pdf>

**Men who have sex with men

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Note. From Changes in HIV Preexposure Prophylaxis Awareness and Use Among Men Who Have Sex with Men — 20 Urban Areas, 2014 and 2017 [Chart], by Centers for Disease Control and Prevention, 2019, Centers for Disease Control and Prevention. <https://www.cdc.gov/mmwr/volumes/68/wr/mm6827a1.htm>

References

Finlayson, T., Cha, S., Xia, M., Trujillo, L., Denson, D., Prejean, J., Kanny, D., Wektert, C., & National HIV Behavioral Surveillance Study Group. (2019). Changes in HIV Preexposure Prophylaxis Awareness and Use Among Men Who Have Sex with Men — 20 Urban Areas, 2014 and 2017. *Morbidity and Mortality Rates Weekly Report*, 6 (27), 597-603. <https://www.cdc.gov/mmwr/volumes/68/wr/mm6827a1.htm>

NIAID. (2016). Pre-Exposure Prophylaxis (PrEP). National Institute of Allergy and Infectious Diseases. https://commons.wikimedia.org/wiki/File:Pre-Exposure_Prophylaxis_%28PrEP%29_2832514377531%29.jpg